



AISH adjudication guide

The Assured Income for the
Severely Handicapped (AISH)
program



Contents

The AISH program.....	3
Eligibility determination	3
Medical eligibility adjudication	3
AISH application process	4
Medical adjudication	5
Part 1 - “severe handicap” adjudication	6
Component 1 – Does the applicant have an impairment of mental and/or physical functioning?	6
Component 2 – Does the impairment cause a substantial limitation in the applicant’s ability to earn a living?.....	9
Component 3 – Is the impairment likely to continue to affect the applicant permanently, because no remedial therapy is available, which would materially improve the applicant’s ability to earn a living?.....	13
Part 2 – approval of benefits	17
Analysis of Reasonable Employment	17
Part 3 – other considerations	19
Internal Referrals and Medical Consultations	19
Conclusion	21

The AISH program

The Assured Income for the Severely Handicapped (AISH) program provides financial and health benefits to eligible adult Albertans with a permanent medical condition, which prevents them from earning a living.

Eligibility determination

AISH has sole responsibility for determining whether an applicant meets medical, financial, age and residency eligibility criteria for the program. These criteria are adjudicated as the application is processed. AISH staff include:

AISH Assessors trained to conduct an initial review of the AISH Application to ensure the necessary information and supporting documentation are included. They will contact applicants and/or physicians to coordinate and collect anything that is missing. They also determine general eligibility (age and residency).

AISH Adjudicators consider the Applicant Information, Medical Report, and supporting documentation for medical adjudication. They have extensive training in AISH legislation, case law and policies, as well as health sciences knowledge and access to medical information and expertise. AISH Adjudicators assess an applicant's situation holistically and make a medical eligibility determination that is guided by facts and bound by legislation. If needed, they will request medical opinions from contracted medical experts.

AISH Generalists determine financial eligibility by considering income and asset information and supporting documents provided with the AISH Application.

Medical eligibility adjudication

This AISH Adjudication Guide outlines the approach AISH staff follow to determine medical eligibility for the AISH program. To be medically eligible, the applicant must have a medical condition that meets the criteria for a "severe handicap" as defined in the AISH Regulation.

In some cases, where the applicant is palliative or placed in a Continuing Care Home, the application is prioritized for immediate review. The applicant may be considered medically eligible for the program as soon as AISH receives documentation validating their medical situation. This information can be provided by the applicant or physician as a letter, or other documentation, and submitted in lieu of the Medical Report.



AISH application process

There are multiple steps to process an application and determine whether an applicant is eligible for the AISH program and benefits.

1. The applicant submits an AISH Application.
2. An AISH Assessor conducts an initial review to ensure all sections of the application are complete (Application Information and Medical Report) and that mandatory documents and signatures are included. If information or documents are missing, the applicant and/or physician are contacted and informed of how to submit the information.
3. Once the application is considered complete, the AISH Assessor reviews the application for general eligibility (age and residency).
4. If the applicant meets general eligibility, the application is forwarded to an AISH Adjudicator for medical adjudication.
5. The AISH Adjudicator assesses the applicant's medical eligibility for the program. If the AISH Adjudicator needs more information, or there is an indication the applicant's disability affected the way they completed the application, follow up will occur with the applicant, their advocate and/or physician.
6. Once the AISH Adjudicator has the required information, they complete the medical adjudication process and approve or deny the applicant's medical eligibility.
7. If medically approved, an AISH Generalist reviews the Applicant Information and the submitted documentation relating to income and assets for the applicant and spouse (if applicable) to determine financial eligibility. The AISH Generalist may request additional information, if required.
8. A letter of approval or denial is sent to the applicant notifying them of the decision.
9. If the application is approved, the AISH Generalist will contact the applicant by phone or mail to schedule a meeting to finish the application process.
10. If the application is denied, an AISH staff member notifies the applicant, and provides the options available if the applicant disagrees with the decision.
11. If at any point the applicant does not meet general or medical eligibility criteria, financial eligibility criteria is not assessed.



Medical adjudication

AISH Adjudicators complete an Adjudication Summary to document the review process. A complete Adjudication Summary includes:

- A synopsis of the AISH Application Medical Report content provided by the physician and/or specialist(s). It may include details from the physician, diagnosis(es), medical history, specialist involvement, hospitalization(s), degree of impairment (physical, mental, cognitive, cumulative effect), medication(s), treatment, and prognosis.
- A review of medical documentation such as: specialist assessments/consultations, hospital reports, discharge summaries, psychological testing (e.g. MOCA, MME), diagnostic reports (e.g. X-ray, ultrasound, MRI), laboratory tests (e.g. blood, biopsy), rehabilitation reports (physiotherapy, occupational therapy), and specialty clinic and/or program reports (e.g. chronic pain, infectious disease).
- A review of the applicant's responses in the Applicant Information sections on employment history and education/ training history.
- The rationale for the AISH Adjudicator's eligibility decisions.

Part 1 - “severe handicap” adjudication

For medical adjudications, AISH decides if an applicant has a “severe handicap” based on the definition in the AISH Regulation, section 1(1)(d). Three components must be satisfied to confirm the applicant meets the “severe handicap” definition:

1. Does the applicant have an impairment of mental and/or physical functioning?
2. Does the impairment cause a substantial limitation in the applicant's ability to earn a living?
3. Is the impairment likely to continue to affect the applicant permanently, because no remedial therapy is available that would materially improve the applicant's ability to earn a living?

Component 1 – Does the applicant have an impairment of mental and/or physical functioning?

Determining whether an applicant has an impairment of mental, cognitive¹ and/or physical functioning is based on the AISH Adjudicator's assessment of the medical information and supporting documentation provided by the physician and/or specialist and the applicant.

Analysis of medical information and supporting documentation

Each application is assessed holistically and on a case-by-case basis. AISH Adjudicators conduct a thorough analysis of the medical information and supporting documentation to make a determination of level of impairment. An applicant may have multiple diagnoses that present varying degrees of impairment. Identifying all impairments is crucial to adjudicate the application.

Medical opinions that include statements such as “the applicant has a disability” or “the applicant has substantial impairments,” do not determine whether or not an applicant has an “impairment of mental and/or physical functioning.”

Analysis of impairment of mental or physical functioning

Diagnosis(es) may be an indicator an impairment is present; however, the applicant must submit sufficient medical evidence to demonstrate how the impairment severely influences mental, cognitive and/or physical functioning. When assessing impairment, AISH Adjudicators review the AISH Application Medical Report and supporting medical documentation to consider:

- diagnosis(es) (primary, secondary, tertiary) and supporting medical information
- duration and prognosis of the diagnosis(es) (date of onset, progression of the diagnosis, stability of the diagnosis with ongoing management)
- relevant medical history and hospitalizations related to the diagnosis(es)
- degree of impairment (physical, mental, cognitive) and effect on the level of functioning
- referral/involvement with specialists (consultations, regular monitoring/follow up, as needed)
- other reports/consultations/assessments confirming the diagnosis(es), diagnostic results, and recommendations (e.g. specialty clinic/programming, diagnostic reports, laboratory tests, hospital discharge summaries, psychological assessments, progress notes, etc.).

AISH Adjudicators conduct a review of the application and supporting medical information to determine if there is a correlation between the applicant's level of impairment and the medical diagnosis(es). They also consider if there are aspects of co-morbidity. This is important, as there may be factors that influence prognosis, which are not only related to a diagnosis, and may impact the severity of the resulting impairment(s).

¹ While the AISH Regulation definition of “severe handicap” refers to mental and/or physical functioning, AISH Adjudicators also examine cognitive functioning as a distinct subset of mental functioning. Cognitive functioning is pertinent in determining if the applicant has the capacity to process, respond, and apply prescribed therapeutic interventions provided by physicians/specialists and/or psychiatrists, or if therapy will only be supportive in nature due to the cognitive functioning of the applicant.

Case Scenarios²– diagnoses, resulting impairments and co-morbidity

Case Scenario 1: two or more diagnoses may cause the same impairment

The symptom of shortness of breath (dyspnea) may be caused by both chronic obstructive pulmonary disease (COPD) and congestive heart failure (CHF). Both could result in a substantial impairment, because of the amplifying effect between the two diagnoses. With treatment, shortness of breath associated with COPD may typically be controlled; however, this treatment may not result in resolving symptoms associated with CHF.

ANALYSIS: There is a physical impairment resulting from two diagnoses.

Case Scenario 2: one diagnosis may result in multiple impairments to different body systems

Diabetes mellitus may produce multiple impairments such as reduced vision due to diabetic retinopathy, sensory issues in the legs due to diabetic neuropathy, skin ulcerations and poor wound healing, renal insufficiency and fatigue. In this case, the diagnosis has progressed resulting in a severe impairment involving multisystem changes.

ANALYSIS: There is a physical impairment involving different body systems.

Case Scenario 3: two diagnoses within the same body system or body part could interact to cause impairment to that body system or body part

Moderate osteoarthritis of the left ankle after a poorly healed fracture and peripheral vascular disease that results in claudication of the legs could interact, causing an amplification of pain and impairment in ambulation.

ANALYSIS: There is a physical impairment within the same body system.

Case Scenario 4: impairment of multiple diagnoses may interrelate and compound

Morbid obesity, depression, osteoarthritis of the right hip and knee, and chronic kidney disease may result in a compounding interrelationship between the impairment, symptoms and corresponding treatment for each of the diagnoses. These may include: functional limitation resulting from pain of the right hip and knee, restrictions in pain control due to medications that interact with chronic kidney disease, lack of motivation and fatigue from depressive symptoms compounded by restrictions in ambulation reflective of obesity.

ANALYSIS: There is a mental and physical impairment.

²This document contains examples and case studies to assist the AISH Adjudicator. The examples are based on actual cases or combinations thereof, with personal identifiers changed to protect the applicant's anonymity. The case studies are hypothetical, do not depict an actual applicant's experience, and are provided to illustrate various medical conditions. An individual applicant's experience may vary based on his or her individual circumstances.

Examples – assessing the type of impairment

Example 1: assessing an impairment of mental functioning

An applicant is diagnosed with schizophrenia. The applicant is prescribed injectable antipsychotic medication, is monitored by a psychiatrist, has a psychiatric history of hospitalizations, continues to exhibit negative symptoms despite medication, has limited self-care, and a sporadic work history. The psychiatrist has noted a significant impairment and a guarded prognosis.

ASSESSMENT: The applicant has an impairment of mental functioning.

Example 2: assessing an impairment of cognitive functioning

An applicant's psychological assessment confirms a mild developmental disability. The applicant's full scale IQ (FSIQ) is 75 and there are difficulties with attention deficit hyperactivity disorder (ADHD) symptoms. Academic information indicates a wide variability in the applicant's skills. An emotional assessment identifies issues with depression, anxiety, anger, impulsivity and ongoing issues with relationships. There are significant issues with working memory and low education achieved in modified programming. Based on the applicant's history and multiple barriers including ADHD, depression, anxiety, limited social skills and learning disability, compounded by the low FSIQ, the applicant would not materially benefit from psychological treatment and remedial therapy options. Supportive treatment focused on adaptive functioning and life skills is appropriate.

ASSESSMENT: The applicant has an impairment of cognitive functioning.

Example 3: assessing an impairment of physical functioning

An applicant is diagnosed with degenerative disc disease and chronic back pain, and there is an X-ray showing mild degenerative disc disease. The applicant is reported to no longer be able to participate in physical labour. The physician has not referred them for further treatment, and they are taking over-the-counter pain medication. The physician reports that the impairment is moderate as the applicant can no longer continue with current employment.

ASSESSMENT: There are functional limitations; therefore, the applicant has an impairment of physical functioning.

Example 4: assessing an impairment of both mental and physical functioning

An applicant is diagnosed with hypertension, asthma, diabetes, depression and generalized anxiety disorder. Hypertension and asthma are managed with treatment. There is progression of diabetes with neuropathy of the feet, leg ulcers that are not healing, and retinopathy. The applicant has previously been hospitalized, and involved with psychiatric care for five years, where the symptoms are treatment refractory. There is an impairment of physical restrictions related to the progression of diabetes, and the symptoms of depression and anxiety are not resolved despite remedial therapy.

ASSESSMENT: The applicant has an impairment of mental and physical functioning.

Component 2 – Does the impairment cause a substantial limitation in the applicant’s ability to earn a living?

Once an impairment of mental, cognitive and/or physical functioning is established, the AISH Adjudicator examines the applicant’s current situation to determine if currently, the impairment causes a substantial limitation in the applicant’s ability to earn a living.

Direct relationship between impairment and limitation

The application must demonstrate a direct causal relationship between the impairment and the substantial limitation in the applicant’s ability to earn a living. If an applicant is unable to work due to reasons unrelated to their impairment, this will not fulfill the causal relationship requirement. Applicants must have mental, cognitive and/or physical limitations that affect their ability to earn a living to be AISHeligible.

Examples – relationships between the impairment and the limitation

Example 1: indirect relationship between the impairment and the limitation

An applicant is diagnosed with sciatica (back pain) resulting in periodic pain that causes an inability to bend and remain seated for long periods of time. Additional limitations reported a lack of work experience and difficulty reading.

ASSESSMENT: Additional limitations are not a direct effect of the impairment resulting from sciatica.



Example 2: direct relationship between the impairment and the limitation

An applicant is diagnosed with osteoarthritis of the shoulders, degenerative disc disease and moderate spinal stenosis with a history of decompression spine surgery. The resulting impairment includes functional limitations with a limited range of motion of the spine and both shoulders, pain with ambulation, an inability to lift, trouble sitting, issues with personal care, and difficulty walking.

ASSESSMENT: The resulting imitations in the applicant's ability to earn a living are directly related to the impairments associated with the diagnoses.

Ability to earn a living

The AISH Act and Regulation do not define “earn a livelihood” (living). AISH Adjudicators consider both the ordinary meaning and common law interpretation of the phrase³ when analyzing whether an applicant’s impairment is substantially limiting the applicant’s ability to earn a living.

“Earn a livelihood” refers to an applicant’s ability to independently provide for the necessities of life. It means earning enough income to support oneself and, if applicable, dependent family members. Courts have provided guidance on “earn a livelihood,” which can be applied in the AISH context⁴:

- Earning a livelihood” means achieving a threshold income adequate for proper maintenance and support. The means by which that threshold is met are irrelevant.
- The phrase “earn a livelihood” is broad language. It does not state that an applicant must be gainfully employed, and it does not refer to an applicant’s trade or occupation. The language is broad enough to encompass any means by which an applicant can earn a living.
- The meaning of “earn a livelihood” is not to be viewed in the context of any given point in time, but as a reflection of an applicant’s lifelong economic activity and status.

Analysis of a substantial limitation in ability to earn a “livelihood”

The AISH Adjudicator determines whether the diagnosis(es) and resulting impairment is causing a substantial limitation in the applicant’s ability to earn a living. They will also determine a baseline regarding the applicant’s ability to earn a living by considering the applicant’s:

- Highest level of education completed (e.g. high school, special/modified programming, post-secondary, trade school, etc.). The AISH Adjudicator may also indicate the reason why the applicant did not complete the educational program, if relevant
- training opportunities pursued and/or involvement in employment training
- reported employment, dates of employment, part-time/full-time, type of work, and reason for leaving
- current work situation. If the applicant is still working, the AISH Adjudicator will determine whether the current work arrangement should be considered “earning a livelihood.”

The AISH Adjudicator will then consider if and how the mental, cognitive and/or physical impairment impacts the applicant’s ability to earn a living. This can include factors such as medical limitations or medical requirements that limit the time available to earn a living, such as treatment and associated programming or medication(s) use that may affect functioning.

³For example, *Petrowski v. Petrowski Estate*, 2009 ABQB 196, *Soule v. Johansen Estate*, 2011 ABQB 403, *Soto v. Canada (Minister of Citizenship and Immigration)*, 2002 FCT 768.

⁴*Petrowski v. Petrowski Estate*, 2009 ABQB 196, at paras. 501 and 502.

Pattern of work

An applicant's pattern of work, such as gaps in employment history is requested in the employment sections of the Applicant Information. It can be a useful indicator of an applicant's capacity to work. When this information is examined with the medical information, it may show a pattern of work that is consistent with their mental, cognitive and/or physical limitation.

The type of work an applicant has done previously reflects the physical and mental capacities required in those jobs. This includes the degree of physical work (heavy to sedentary work) and mental aptitudes. Previous job experiences may inform the employment options for applicants. Generally, there are more physical barriers to returning to heavy work than to sedentary occupations. Skills acquired in a previous job(s) may enable an applicant to find other work that is suitable to their limitations.

When there is no current work or other work-related activity indicated in the application, the AISH Adjudicator must rely on past work activity to make a decision. When there is current work activity, it must be considered in the "earning a livelihood" determination.

Part-time employment may not be indicative of an applicant's work capability, but rather may be a matter of personal choice.

If an applicant has worked regular periods of time in the past, followed by periods of limited or no work activity, this may indicate a progressive, episodic and/or cyclical medical condition. Medical reports related to dates and reasons for the applicant's visits to their physician(s) may be pertinent. When an applicant's previous employment is seasonal, unemployment periods during the "off season" may not be indicative of their capability to earn a living.

By considering the applicant's type of employment and the pattern of work activity, the AISH Adjudicator determines whether the applicant's past work experience is relevant to their capacity to earn a living.



Other considerations

Applicants may have had an inclusive employment arrangement, such as extensive work accommodations that allowed them to perform a job. This type of position should not be indicative of the applicant's ability to earning a living. Determining if the applicant has capacity for earning a living requires assessing the relationship between the medical condition(s), the current and past work positions, and the applicant's personal situation.

Volunteer positions are often referred to as "volunteer work." The fact an applicant participates in volunteer work may not, in itself, indicate a capacity to earn a living. The AISH Adjudicator must determine if the nature and degree of volunteer activity indicates capacity for paid employment. When determining initial eligibility for AISH, individuals who engage in volunteer activity are considered applicants who are not working.

An applicant can stop work for many reasons, which may or may not relate to their disability. The determination of eligibility for AISH benefits is based on an applicant's capacity to work relative to whether there is a substantial impairment caused by mental, cognitive and/or physical limitation(s). An applicant's limitation must regularly prevent them from working or limit the amount they can work at an occupation they could normally pursue.

Example – substantial limitation in ability to earn a living

An applicant is diagnosed with autism, and the impairment is described as mild by the physician. A psychological assessment has noted that while their full scale IQ is 95, there are significant social and communication issues and poor adaptive skills functioning. The applicant required modified school programming and received supports since early childhood. Compounding this, they developed generalized anxiety disorder in early adolescence. The applicant has a limited work history stocking shelves at a grocery store, and they were let go due to poor work performance. The applicant had difficulty following directions, interacting with the supervisor and staff, and would leave the worksite without notice due to anxiety related symptoms.

ASSESSMENT: The impairment causes a substantial limitation in their ability to earn a living.

Component 3 – Is the impairment likely to continue to affect the applicant permanently, because no remedial therapy is available, which would materially improve the applicant's ability to earn a living?

This component looks to the future to understand if any remedial therapy options exist that would materially improve an applicant's ability to earn a living.

Analysis of permanence

The AISH Adjudicator must review relevant information reported by the applicant and the physician/specialist(s) to assess the likelihood the applicant's impairment will continue into the future and impact their ability to earn a living. A determination of permanence is made by identifying the diagnosis(es), examining the severity of the impairment, the limitation(s) caused by that impairment and reviewing remedial therapy options available, which may lessen the impairment.

Episodic, cyclical and/or recurring medical conditions
Episodic and/or cyclical medical conditions may recur given their nature. These medical conditions may have greater complications, which could have an impact on an individual's ability to work. Each recurrence impacts the applicant's treatment and recovery, and may result in decreased functioning over time. There may also be shorter intervals between relapses, more invasive treatment and/or longer work absences. The cumulative effect of episodic and/or cyclical conditions may result in a permanent disability that will continue to affect the applicant's ability to earn a living.

Progressive medical conditions

Progressive medical conditions are diagnoses that advance in scope and worsen in severity over time. There is often no cure, and they affect older adults, when characteristic signs and symptoms become more prominent. A progressive disease may be considered rapidly progressive (days to weeks) or slowly progressive (months to years). Numerous progressive

diseases and related symptoms may be managed or delayed with treatment and remedial therapy options. As the medical condition or symptoms progress, there may be treatment and remedial therapy options available that will delay progression and/or address symptom control. These types of disabilities resulting from progressive diseases may require modifications over time, as the need for support changes. This may include modifications to medical treatment, health-related supports, workplace accommodations and/or equipment and aids. Being diagnosed with a progressive medical condition may not result in an applicant's inability to earn a living. The AISH Adjudicator must assess whether the medical condition is progressing rapidly or slowly, any treatments available to slow progression and address symptoms, and modifications to support activities associated with daily living and the work environment.

Examples – progressive medical conditions and ability to earn a living

Example 1: progressive medical condition managed

An applicant is diagnosed with arthritis that resulted in considerable functional physical limitations causing a severe impairment. The treating physician initially treated them with nonsteroidal anti-inflammatory drugs; however, the applicant was not responding to treatment and functional limitations continued to progress. The applicant was re-assessed and diagnosed with rheumatoid arthritis (RA). Since the applicant was experiencing an acute flare up and not responding to previous treatment, the specialist prescribed a non-biologic disease modifying anti-rheumatic drug (DMARD). This medication improved physical functioning and decreased pain. The applicant was able to return to work as an Administrative Assistant.

ASSESSMENT: New medication resulted in improved functioning and lessened the severity of the impairment, so the applicant no longer had a substantial limitation to earning a living.

Example 2: progressive medical condition ongoing

Another applicant diagnosed with RA used the same treatments, but the medical condition continued to progress, resulting in ongoing swelling due to inflammation in the hands. The applicant required an additional biologic response modifier (newer class of DMARDs) and surgery to address severe joint damage in the knees. This treatment and remedial therapy did not result in improved functioning to support daily activities.

ASSESSMENT: The applicant has a substantial limitation in their ability to earn a living.

Analysis of remedial therapy

Remedial therapy includes any therapeutic options that are prescribed or recommended by a physician or specialist and may include medical treatments such as prescription medication, surgery, physical and occupational therapy, psychotherapy and the use of medical devices such as a wheelchair, Continuous Positive Airway Pressure [CPAP] machine, pacemaker, insulin pump, cochlear implants, etc.

It also includes non-medical therapy and remedies such as art therapy, psychoeducational materials and tools, self-help resources, meditation/mindfulness and other mind-body practices, yoga, natural products, lifestyle changes, the use of therapy animals and job supportive measures to achieve a satisfactory function and skill level.

Remedial therapy and therapeutic options vary depending on the nature and severity of the diagnosis(es) and the applicant's response to treatment. In some cases, the intent is to cure or remove the cause of the medical condition. In other cases, the intent is to control the medical condition from progressing, relieve symptoms, improve functional capabilities, and/or provide the applicant with insight and necessary coping mechanisms to adapt to their limitations.

The AISH Adjudicator must review information in the application to assess an applicant's involvement in, and response to, remedial therapy or therapeutic options. They consider how an applicant's current therapies impact their ability to earn a living. They also use their knowledge of

the health care sector and available supports to determine whether there are other therapies available to help improve the applicant's medical condition and ability to earn a living.

The AISH Adjudicator considers remedial therapy, which is inclusive of medical and/or psychological information, the applicant's self-reported information, and includes the following:

- duration and prognosis of the diagnosis(es) (date of onset, progression of the diagnosis, stability of the diagnosis with ongoing management)
- pending referrals
- involvement in treatment and therapy (previous, current and future treatment and therapy)
- modality of therapy/treatment
- availability of therapy/treatment
- duration of therapy/treatment (short -or long-term, indefinite)
- stage of treatment (recent diagnosis, referrals pending, recommendations recently initiated, full engagement in treatment, and if an appropriate time has lapsed to determine the response to treatment)
- response to therapy/treatment
- factors that may be impacting response to therapy/treatment (e.g. compliance, co-morbidity, psychiatric, social issues such as medication coverage, etc.)
- other potential medical and non-medical treatment and remedial therapy options that may not be listed, but are often associated with the diagnosis(es)
- use of medical devices and/or assistive devices
- whether treatment is used for stabilization, ongoing management to maintain current level of functioning, to improve functioning, or being used as a palliative measure
- individual effort made to correct, control or lessen the severity of the impairment caused by the diagnosis(es)
- ability to follow therapy/treatment recommendations
- medical limitations to earning a living, such as medical requirements which limit the time available to earn a living (treatment and associated programming) or use of medication(s), which may affect functioning.

Remedial therapies timing is also important. What stage of treatment an applicant is in will assist an AISH Adjudicator to determine if all remedial therapy and treatments have been exhausted, leading to a permanence determination. Completion of treatment and ongoing management of a diagnosis may result in the resolution of, or lessening of, the impairment, which may support the applicant in earning a living. The stage of treatment is pertinent in determining AISH eligibility. The applicant's symptomology may evolve with the use of various medications that support recovery. It may take time to establish rapport with health professionals and achieve an optimal level of treatment and participation by the applicant, as they process their diagnosis. These factors can affect the impact the identified treatment plan will have on the prognosis and ability to earn a living.

Job supportive measures may also be inclusive of assistive devices that complement remedial therapy approaches. These assistive devices may support independence and inclusion in work settings, which may not necessarily require involvement with a medical professional. Assistive devices may support a number of medical conditions including:

- mobility (e.g. cane, brace)
- vision (e.g. magnification devices, task lighting)
- mental health and cognition (e.g. technology such as handheld personal devices/adaptive computer technology/apps available online, or built in accessibility software to provide schedules/reminders to support memory and executive functioning, provide distraction from anxiety and agitation, monitoring of fitness, breathing, and sleeping)
- dexterity (e.g. hands free wireless communication tools, adaptive helping tools, electrical appliances)
- hearing loss (e.g. hearing aids, alerting device, tele-printer/tele-typewriter)

These treatments, therapies and assistive devices may positively impact the applicant's level of impairment related to mental, cognitive and/or physical functioning.

Example of an assistive device which would materially improve an applicant's ability to earn a living

Example: assistive device

An applicant is considered legally blind and requires accommodations at work. The applicant works as an office assistant, which requires interaction with clients, answering phone calls, scheduling for staff, booking appointments, and data entry. Assistive devices used at work include the following:

- assistive technology, such as computer screen magnification software (changes the font size, shape, enlarges icons, enhances the mouse pointer and changes the screen colours)
- screen reading software (supports the reviewing of electronic file materials to complete data entry)
- handheld portable note-taker (can electronically receive, store and retrieve data to support meeting minutes)
- anti-glare guard (reduce glare from the computer screen)
- task lighting and a large print telephone (has colour labels to identify keys and phonelines)

It is also noted, the applicant uses a cane to support getting to work, and when required an electronic aid (e.g. personal smart phone), which provides auditory mapping locations as required.

Examples of whether or not remedial therapy is available, which would materially improve the applicant's ability to earn a living

Example 1: treatment and remedial therapy

An applicant was diagnosed with depression, ended their previous employment as a server, and reports they left this position due to stress. The applicant recently began using medication and was provided with a note to be off work for three months. Permanency as it applies to the applicant's ability to earn a living has not been confirmed.

The applicant recently initiated treatment for depression and did not require a referral to a psychiatrist and/or further counseling. The medical note indicates a three-month duration for treatment. Treatment and remedial therapy may materially improve the applicant's ability to earn a living.

Example 2: treatment and remedial therapy

An applicant was diagnosed with Chronic Pain Syndrome and has stopped their previous employment as a heavy equipment operator due to pain. It is noted, the applicant has been off of work for two years, involved with regular follow ups with their physician, has participated in active rehabilitation through a pain clinic and treatment is regarded as maintenance only. The physician has confirmed the applicant's condition is chronic and the resulting impairment will not improve. There is no treatment or remedial therapy, which will materially improve their ability to earn a living.

Example 3: stage of treatment

An applicant was diagnosed with Major Depressive Disorder. The applicant was initially diagnosed by the physician, and prescribed an antidepressant. No further treatment was pursued for a period of four months. Due to a lack of response from the antidepressant, the treating physician referred the applicant to a psychiatrist. Upon assessment, the psychiatrist diagnosed the applicant with Dysthymia, Generalized Anxiety Disorder, and Borderline Personality Traits. The psychiatrist changed the medication to a different antidepressant, added an anxiolytic, and a small dose of an antipsychotic medication. The psychiatrist referred the applicant to a mental health therapist to participate in Cognitive Behavioral Therapy (CBT), mindfulness to address stress, and recommended attending group therapy at the day program offered at the local hospital. The applicant has an impairment of mental functioning; however, there is remedial therapy available, and the applicant recently initiated psychiatric treatment. The treatment outlined may materially improve the applicant's ability to earn a living. As the applicant is in the early stages of treatment, neither prognosis nor response to treatment can be confirmed.

Part 2 – approval of benefits

After determining an applicant has a “severe handicap,” AISH Adjudicators review the application to decide whether the applicant will be approved for benefits based on employment/employability criteria under sections 5(1)(b)(i) and 5(1)(b)(iii) of the AISH General Regulation. Benefits may not be approved if the applicant has:

- refused to seek, accept, or has terminated reasonable employment, or
- failed to make use of appropriate training or rehabilitative measures

Even if an applicant's mental, cognitive and/or physical limitations are considered permanent, because there is no remedial therapy available to materially improve their ability to earn a living, there may be alternative employment, training or rehabilitative opportunities, which would allow them to work despite the permanence of their limitations.

AISH Adjudicators have authority to exercise discretion under these AISH Regulation sections when deciding whether to approve or not approve benefits. They consider relevant information and circumstances including:

- highest level of education completed, such as high school, special/modified programming, post-secondary, trade school, etc. (the AISH Adjudicator may also note the reason why the applicant did not complete the educational program, if relevant)
- all reported employment, dates of employment, part-time/full-time, type of work, and reason for leaving
- other steps taken to seek employment suitable to the medical condition (including other types of employment/training if the medical condition prevents the applicant from returning to their previous employment)
- additional rehabilitation/training opportunities pursued and/or involvement in employment training
- whether the applicant is currently involved with school or training and the tentative completion date

Analysis of Reasonable Employment

Reasonable employment” refers to an occupation in which an applicant may reasonably be expected to be employed, because of their work experience, skills, education and/or training. It may also refer to the capacity to acquire the necessary skills, education or training, on the job or otherwise, given the applicant's limitations. In some instances, an individual may have worked at a highly skilled position, such as an engineer out in the field, and can no longer continue in that position due to physical limitations. However, the individual has the capacity to work as an engineer at a desk job.

Examples – refused to seek, accept or terminated reasonable employment

Example 1: applicant not exploring opportunities

An applicant is a hairdresser by trade and can no longer complete job duties due to spinal stenosis. The impairment includes functional limitations in standing for long periods. The applicant has not explored other employment opportunities that reflect their current physical restrictions.

ASSESSMENT: The applicant has refused to seek reasonable employment.

Example 2: applicant not exploring opportunities

An applicant worked as a cashier for the past two years and quit their job six months ago. The applicant has a diagnosis of irritable bowel syndrome and dysthymia. The applicant has been prescribed medication and is receiving no other treatment. Both medical conditions have been longstanding and the applicant maintained employment for the past four years. Since quitting, the applicant has not taken any steps to look for employment.

ASSESSMENT: The applicant has terminated employment and refused to seek reasonable employment.

Example 3: applicant considering training

An applicant has recently completed high school. While attending school, they worked part-time at a fast food restaurant, then quit after graduation. The applicant may pursue culinary arts post-secondary education next year. The applicant has a diagnosis of attention deficit hyperactivity disorder (ADHD), generalized anxiety disorder (GAD), a mathematics learning disorder and a full scale IQ of 89. They use a variety of substances including alcohol and marijuana. The applicant is actively involved with psychiatric follow up, using medication to address symptoms, and has been referred for substance abuse treatment and rehabilitation to support ongoing abstinence. During high school, the applicant used learning accommodations to support their graduation. Based on their age, involvement with treatment, additional substance abuse treatment recommendations, and their previous ability to participate in work/school, the applicant's capacity to earn a living cannot be confirmed, as there is not a substantial limitation identified. The medical condition is considered stable, based on the current treatment plan and treatment recommendations for substance use are being addressed.

ASSESSMENT: The applicant has not sought reasonable training and/or employment reflective of their current abilities, skills and limitations.

Analysis of appropriate training or rehabilitative measures

Appropriate training or rehabilitative measures include potential education, training, and/or other activities that would increase an applicant's ability to earn a living.

Retraining or going back to school are two options, which may enable an applicant to acquire the necessary skills and education to rejoin the workforce. The AISH Adjudicator must assess the applicant's demonstrated suitability and aptitude for a different occupation in conjunction with the diagnosed medical condition(s) and resulting impairment. While education or training options may be available, they may not be suitable for the applicant. If this is the situation, the AISH Adjudicator would not conclude the applicant has not made use of appropriate training.

Example – appropriate use of training or rehabilitative measures

An applicant has a history of a crush injury to the right foot. After a period of treatment and rehabilitation, it was determined they could not return to their previous employment as a server. The physician confirmed an ongoing impairment with ambulation issues related to the right foot, and that the applicant would continue to have a lifelong physical restriction. The physician recommended the applicant look for sedentary employment to accommodate their physical limitation and consider employment training to connect with this type of work. The applicant has a grade 12 education and no indication there are any issues with learning. At the time of the application, the applicant had not pursued the physician's recommendations.

ASSESSMENT: The applicant failed to make use of appropriate training and rehabilitative measures that would be reflective of their current impairment.

Part 3 – other considerations

Internal Referrals and Medical Consultations

Internal referral for specialized assessments Information related to physical and psychiatric assessments is often provided through the medical documentation.

Occasionally, the AISH Adjudicator may authorize a referral for a specialized assessment to support the decision-making process and to clarify provided medical information.

Specialized assessments may also be authorized when medical information is not available or accessible by an applicant, or when medical complexities are described in the application. Specialized assessments may comprise a combination of one or more assessment types including:

- Vocational Assessment – identifies the potential for competitive employment by:
 - evaluating intellectual functioning, academic potential, vocational aptitudes and career interests—tests vary and are dependent on the applicant
 - identifying specific barriers to competitive employment, which are indicated by the applicant and presented in medical documentation and other information that is available
 - considering employment-related rehabilitation and retraining that may be appropriate
 - providing recommendations for overcoming barriers to employment
- Psychological/Disability Assessment – identifies the current disability and the effect it is having on the applicant's life by:
 - looking at the limitations experienced due to their medical condition and/or impairment, and determining how they are coping with the limitations
 - examining the individual's psychological and emotional functioning
- Neuropsychological Assessment – is used with applicants who have sustained a traumatic brain injury or have a brain injury of non-traumatic origin. Tests may vary and are dependent on the applicant. The assessment identifies the nature and extent of any neuropsychological problems, impairments and disabilities. It also provides a prognosis and recommendations for improving function by:
 - identifying past and present intellectual functioning, attention and working memory, verbal memory, memory for visual material and prospective remembering, problem solving, executive functioning, and visual, spatial and perceptual skills, expressive and receptive language abilities, and somatosensory and motor function
 - assessing personality, behavioral abilities and emotional factors, if needed
 - evaluating cognition and behavior by examining the effects of a brain injury or neuropathological process the applicant may have experienced.
- Functional Capacity Evaluation – examines physical capability for work by:
 - completing a manual examination, physical examination and assessing overall strength capabilities
 - reviewing the applicant's ability to tolerate the physical demands of work
- Chronic Pain Assessment – it determines how pain affects the applicant through tests that vary and are dependent on the applicant, and by:
 - providing a detailed analysis of the applicant's history of pain and pain-related treatment, use of opioid medications, current perception of pain, pain-related disability, coping with pain, and personality/psychological functioning
 - using psychometric measures to determine whether maximum medical improvement has been attained and future prognosis
- Cognitive Assessments – it uses tests that vary and are dependent on the applicant to assess current cognitive functioning, intelligence quotient (IQ), academic achievement level and psychometric assessment of adaptive functioning

Once the completed assessment is returned to the AISH Adjudicator, the assessment is documented in the Adjudication Summary and incorporated into the eligibility decision.

Medical consultation

For particularly complex applications, the AISH Adjudicator may ask a contracted AISH Medical Consultant to review the medical information provided and provide an analysis and opinion. This review may be requested when:

- an applicant is not responding to typical treatment
- there are multiple letters from physicians or specialists with discrepancies or conflicting information
- specialist information does not support the diagnosis, and the treating physician is contradicting the information provided by the specialist
- there are multiple submissions of medical information
- medical reports or diagnostic imaging require interpretation
- there are chronic and concurrent conditions with potential multisystem issues occurring

Once the consultation report is received, the response is documented in the Adjudication Summary and incorporated into the eligibility decision.

A person wearing a dark suit is shown from the chest down, holding and reviewing a large, white document. The document contains several columns of text, which are mostly illegible due to the blue tint and blurring. In the background, a white computer keyboard is visible on a desk. The entire image has a strong blue color overlay.

Conclusion

Upon completion of the assessment of the above criteria and considerations, the AISH Adjudicator determines medical eligibility. Applicants who meet the medical eligibility and employment/employability criteria will then be assessed for financial eligibility.